Shoulder Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 14 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *Examine the normal side first if one side is painful †.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: usually start standing
☐	Exposure: sleeveless top at least (singlet ideal); compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both sides, from front, side and back			
☐	Posture	Shoulder height, position of arm	Guarding, dislocation
☐	Skin and scars	Erythema, bruising, arthroscopy or surgical scars	Infection, previous surgery
☐	Swelling	Over joint, AC joint	Effusion, AC injury
☐	Wasting	Deltoid, supraspinatus and infraspinatus fossae	Rotator cuff tear, nerve injury
☐	Deformity	Step at AC joint, squared-off shoulder, winged scapula †	AC dislocation, glenohumeral dislocation
Feel — start medially and work round to the back			
☐	Temperature	Back of hand over both shoulders	Warm = inflammation, infection
☐	SC joint → clavicle → AC joint	Palpate along the bones	Tenderness, step, swelling
☐	Head of humerus	Anterior and lateral	Tenderness
☐	Long head of biceps	Bicipital groove (arm externally rotated †)	Biceps tendinopathy
☐	Scapula	Borders and spine	Tenderness
☐	Crepitus	Hand over joint while moving it through a limited range	OA, rotator cuff disease
Move — active, then passive if limited (watch from front and back)			
☐	Abduction	"Lift your arms out to the side and up" (0–180°); palpate for crepitus	Painful arc 60–120° = impingement †
☐	Adduction	"Bring your arm across your body"	
☐	Flexion	"Lift your arms forwards and up" (0–180°)	
☐	Extension	"Move your arms backwards" (0–60°)	
☐	Internal rotation	Hand behind back, reach up the spine (note highest level †)	Frozen shoulder, cuff
☐	External rotation	Elbows tucked in at 90°, rotate forearms out (0–60°)	Lost first in frozen shoulder †
☐	Measure	Limited → estimate/measure degrees; otherwise "full range of motion"	
Special tests			
☐	Empty can (Jobe's)	Arm at 90° elevation, 30° forward (scapular plane), thumbs down; push down while patient resists	Pain / weakness = supraspinatus (rotator cuff)
☐	Hawkins (impingement)	Shoulder and elbow flexed to 90°; your arm as fulcrum, internally rotate the shoulder	Pain = subacromial impingement; negative = unlikely
☐	Apprehension (supine)	Shoulder 90° abducted, elbow 90° flexed; gently externally rotate	Pain or apprehension = anterior instability
☐	Clunk test	Supine, arm fully abducted; full external rotation, push humeral head slightly anteriorly	Grinding / clunk = labral tear †
☐	Sulcus sign	Arm at side, neutral rotation; pull down at the elbow	Gap >1 fingerbreadth below acromion = inferior instability

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the neck and elbow (joints above and below) and do a neurovascular exam of the upper limb †."
☐	Present: "On inspection there was no wasting, swelling or deformity. No tenderness or warmth. Full active and passive range of motion without pain. Special tests for rotator cuff, impingement and instability were negative."

4. Quick reference †

Pattern	Key findings
Rotator cuff tear / tendinopathy	Weak, painful abduction; positive empty can; wasting of supra/infraspinatus
Subacromial impingement	Painful arc 60–120°; positive Hawkins
Frozen shoulder (adhesive capsulitis)	Global loss of active AND passive movement, external rotation first
Anterior instability	Previous dislocation; positive apprehension
AC joint pathology	Tenderness/step at AC joint; painful arc at the top (170–180°)